

MINUTES OF THE MEETING OF THE COLLEGE OF INFORMATION TECHNOLOGY INTERVIEW FOR STUDENT LEARNING COMMUNITY

AY 2026-2027

Participants:

ULS Clinic Doctor : **WILFRIDO SIMBUL, MD**

IT Students

1. Current Clinic Processes & Record Keeping

- Patient Handling vs. Record Keeping: Patient care consumes most of the clinic's time.
- Record Management: The clinic uses a hybrid system.
 - Hard copies are maintained at the front reception desk.
 - Soft copies are personally managed by the doctor using Microsoft Excel spreadsheets.
 - Retrieving soft copies on the computer is much faster and allows immediate patient consultation.
- Current Data Storage: Master list of enrolled students (6,367 students across different colleges) and consultation records are categorized manually in Excel.

2. Appointment & Consultation Workflow

- Medical Sickness Cases: No appointment needed. Patients can walk in anytime during office hours.
- Wellness & Scheduled Services: Appointments or set schedules are applied for non-sickness / wellness cases:
 - Dental Checkups: Scheduled per class (e.g., BS IT 3rd Year) due to the dentist working part-time.
 - Vaccinations: Scheduled per group/college (e.g., Engineering students going for off-campus OJT).
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3. Inventory Management System Needs

- Current Setup: Manual logging/listing maintained by nurses outside.
- Key Challenges:
 - Nurses currently have to physically check stock levels, occasionally realizing medications (e.g., Amlodipine, Sinutret) are out of stock or expired during active consultations.
- System Requirements for Inventory:
 - Automated expiration alerts (e.g., highlighting items in red when expired or approaching expiration).
 - Real-time stock level monitoring and low-stock/out-of-stock indicators to assist with timely budget requests and re-ordering.

4. Key System Features & Reporting Requirements

- Automated Data Analytics & Statistics:
 - Graph Generation: Automatic conversion of raw patient/sickness data into visual graphs (to easily spot disease outbreaks like flu, viral conjunctivitis, etc.).
 - Categorization by Health Condition: Automated tallying for various categories such as URI/flu, wounds, neurological issues, mental health conditions (anxiety, depression, panic attacks), and metabolic cases.
 - Automatic Calculated Fields: Automatic computation of Age, Sex, BMI (with color coding for normal, overweight, obese), and Blood Pressure status.
 - Chronological Logs: Separate tracking for general walk-ins/checkups (wellness/validations) versus medical consultations (sickness).
- Demographic & Grouping Categorization:
 - Student Tracking: Ability to filter and generate statistics per college/department.
 - Employee / Faculty Tracking: Grouping employees into distinct categories (Deans, Full-Time Faculty, Part-Time/City Faculty, and Maintenance) due to differing exposure levels and medical terms.
- Date & Patient History Automation:
 - Automated date-stamping when selecting or opening a student record for consultation.

5. Additional Context & Action Items

- Integration Potential (Government Programs): The doctor highlighted the potential alignment with government healthcare initiatives (such as *Yakap* free medicine coverage), emphasizing the need for a robust database system.
- Project Coordination / Collaboration:
 - The doctor suggested that the two student groups (Section A and Section B) consolidate their efforts into building a single, unified system rather than two competing systems.
 - The students agreed to propose the unified approach to their project professors.
 - User Experience Consideration: Keep the UI intuitive and familiar (similar to standard Excel data structures) to minimize user friction for non-tech-savvy clinic staff.
- Next Steps:
 - Students will present system drafts/prototypes to the clinic doctor for continuous checking and feedback prior to graduation.

Prepared by,

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